

# Westview Cancer Center — Oncology Inpatient Unit

## INPATIENT DISCHARGE SUMMARY — ONCOLOGY

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

|                                     |                                                   |                     |
|-------------------------------------|---------------------------------------------------|---------------------|
| <b>Patient:</b> Patricia G. Morales | <b>DOB:</b> 07/03/1971                            | <b>MRN:</b> 5509374 |
| Date: September 30, 2024            | Provider: Dr. Anita Sharma, MD — Medical Oncology |                     |

### ONCOLOGY BACKGROUND

|                           |                                                               |                         |                                                       |
|---------------------------|---------------------------------------------------------------|-------------------------|-------------------------------------------------------|
| <b>Diagnosis:</b>         | Stage IIIA Invasive Ductal Carcinoma, Left Breast (C50.912)   | <b>Receptor Status:</b> | ER+/PR+/HER2- (Luminal B)                             |
| <b>Treatment Regimen:</b> | AC-T (Doxorubicin/Cyclophosphamide x 4, then Paclitaxel x 12) | <b>Current Cycle:</b>   | AC Cycle 3, Day 14 — admitted for febrile neutropenia |
| <b>Admitted:</b>          | September 25, 2024                                            | <b>Discharged:</b>      | September 30, 2024 (5-day stay)                       |

### ADMISSION COURSE

Ms. Morales, a 53-year-old female undergoing neoadjuvant chemotherapy for stage IIIA breast cancer, presented with fever of 38.9°C on day 14 of her third AC cycle. She reported fatigue, mild pharyngitis, and no localizing symptoms. Initial CBC demonstrated ANC of 280 cells/mcL consistent with febrile neutropenia (ANC <500 with fever).

Blood cultures x2 sets, urinalysis, chest X-ray, and pharyngeal cultures were obtained. She was admitted and started empirically on cefepime 2g IV q8h per febrile neutropenia protocol. Blood cultures drawn on admission remained negative at 5 days. Urine culture demonstrated no growth. Chest X-ray showed no pneumonia or effusion. CRP was 148 on admission, trending down to 38 by hospital day 4.

ANC nadir was 180 on hospital day 2, recovering to 1,420 by day 5. Filgrastim (G-CSF) 480mcg subcutaneous daily was administered for 3 days. Antibiotics were de-escalated to oral amoxicillin-clavulanate on hospital day 4. Patient remained afebrile for 48 hours prior to discharge.

### LABS AT DISCHARGE

| Test       | Result | Units     | Reference | Flag |
|------------|--------|-----------|-----------|------|
| WBC        | 4.8    | K/uL      | 4.5-11.0  |      |
| ANC        | 1,420  | cells/mcL | >1500     | L    |
| Hemoglobin | 10.2   | g/dL      | 12.0-16.0 | L    |
| Platelets  | 187    | K/uL      | 150-400   |      |
| CRP        | 38     | mg/L      | <10       | H    |
| Creatinine | 0.9    | mg/dL     | 0.5-1.1   |      |

### NEXT STEPS AND INSTRUCTIONS

AC Cycle 4 is tentatively scheduled for October 14, 2024 (14 days from now), pending ANC recovery and oncology visit. She will be seen in clinic on October 7 for CBC recheck and symptom assessment.

Filgrastim prophylaxis will be added for all remaining cycles to reduce risk of future neutropenic events. Patient was counseled on neutropenic precautions: avoid crowds, raw foods, and persons with active infections. Threshold for ER evaluation: any fever >38.1°C or chills, regardless of other symptoms. She should call the oncology nurse line before presenting to any outside ED.

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